

Title: Medicaid in 2025: The Impact of the “One Big Beautiful Bill Act” (OBBA)

Introduction: As of 2024, nearly 87 million Americans are enrolled in Medicaid, a public health insurance program serving vulnerable populations. Beginning with the Affordable Care Act (ACA) of 2010, many states expanded eligibility to include low income populations, reducing the number of uninsured residents. In July 2025, the OBBA bill introduced sweeping reforms to the Medicaid program’s eligibility requirements, funding structure, overall design, and fiscal sustainability.

Background and Key Components: Eligibility: Medicaid serves individuals with limited financial resources, including pregnant women, children, seniors, and those with disabilities. States have the option to expand eligibility based on federal guidelines. Based on state regulations and federal provisions, some non-citizens or immigrants with specific legal status can also qualify.

OBBA introduces work mandates, eligibility restrictions, and cuts to specific services

Key changes including: semiannual eligibility redeterminations and cost-sharing of up to \$35 per service, removal of coverage for gender affirming care, reduced access for undocumented individuals, and a rollback in specific benefits for older, low income adults. Experts warn cost controls may limit access and Congress reconsider these strict eligibility rules.	Work Requirement: Adults aged 19 year to 64 years are required to complete 80 hours of work per month or approved community service to maintain Medicaid eligibility. Next Steps: Consider reviewing eligibility, arranging home care assistance implementing asset protection strategies. Experts recommend watching policy updates to protect your benefits.
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Financing: The program is funded by federal and state governments, with federal contributions determined by each state’s per capita income through the FMAP method. The ACA extended Medicaid to millions, with federal government covering the majority of associated costs.

Comparison: ACA (2010) vs. OBBA (2025) (Source: CBO, 2025; KFF, 2023)

SN	ACA (2010)	OBBA Law (2025)
1	Simplified eligibility checks & broader access	Strict eligibility rules & more paperwork
2	Focused on increasing national health coverage	Reduce spending, increase state control
3	Cost sharing is minimal for most enrollees	States may apply charges to some low income adults
4	Enhanced federal matching (upto 90% for expansion states)	Funding caps & federal matching reduced
5	States had flexibility in provider payments	Reimbursement now aligned with Medicare rates

Current Concerns and Issues (2025): OBBBA reforms are seen as being part of the drive for cost containment and state autonomy, which introduces new issues of coverage and equity. Millions of people are facing risks of losing coverage because of an increased number of eligibility checks and administrative barriers. Especially rural safety net hospitals are at risk of funding losses due to limited supplemental payments. State budget holes As state dependency on federal funding tightens, the states may be left with budget holes. While the bill claims to price down, it will actually lead to more levels of uncompensated care and deteriorating health. 1. Tax Policy: With an increased income for upper and middle class families - especially when children are involved - lower tax rates shield overtime pay and special income tax credit. Seniors will not have any federal tax on a certain amount of social security income The Child tax credit was raised from \$2000 to \$4000. 2. Cuts to social spending: a) cuts Medicaid, and turns Medicaid into block grant for states. One of these effects is that somewhere in the range of 10-17 million individuals may be left uninsured (particularly non-disabled adults), and B) the eligibility standard for Food Stamps/SNAP recipients will become more stringent. Time limits apply for able bodied adult with no dependents (iii) Regulate Student Loan: Terminates income driven repayment plans. Increases interest rates for federal student loans Cuts forgiveness programs. Budget and Fiscal Impact: The National Deficit: Greatly increased the national debt to between \$3.2 to \$3.4 trillion over the next 10 years. CBO estimates a surplus of \$2 trillion in the year 2026. Hospitals (particularly in the low income rural areas) are afraid of reduction of reimbursements.

Who Is Affected? Winners: Policymakers aiming to cut costs and non expansion states may benefit from leaner Medicaid systems. ***Losers:*** Low income adults, certain immigrants, and rural recipients face the greatest losses from reduced coverage and reimbursement.

Conclusion: The 2025 Medicaid changes are the most significant changes since the Affordable Care Act & aim to save money and work requirements, marks a policy shift toward fiscal restraint, but at the potential expense of access and equity. Achieving balance between financial sustainability and continued coverage for vulnerable populations remains the central challenge for U.S. Medicaid policy.

References:

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AI-Generated Content vs. Original Research: A Comparative Reflection

As per assignment guidelines, I then merged AI created content with original research to analyze Medicaid reforms as of 2025. Each source provided me with a unique insight into how the policies have changed. AI tools (analysis) were useful for summarizing the key changes in Medicaid and generating a comparison between the ACA and OBBBA, however, they often oversimplified important characteristics of these changes and failed to address eligibility limitations, provider taxes, and other key considerations for dual eligible seniors. New CBO, KFF and CMS research provided more accurate data and better context. Issues such as provider payment reductions were only identified in sources from Commonwealth Fund and AHA. OBBBA - AI was also unable to identify vital information about how Medicare rate-based payment is applied to providers or the potential loss of access to some services for dual eligible seniors, the impact on rural hospitals or the specific reasons that encourage Medicaid disenrollment. The Congressional Budget Office (CBO) report, the Kaiser Family Foundation (KFF) report, the CMS report, and the Commonwealth Fund's study were more comprehensive. These sources offered qualitative descriptions of how rollbacks to eligibility redeterminations, paperwork requirements and other regulatory burdens had the potential to result in large-scale coverage losses with a disproportionate impact on low-income adults, immigrants and individuals with chronic health conditions. CBO estimates indicated that more than 70% of disenrollments under OBBBA would be because of administrative problems as opposed to ineligibility, which the AI completely failed to address.

Also, reports by the American Hospital Association (AHA) and The Commonwealth Fund have also reported how the decreased federal funding will destabilize the rural and safety-net hospitals leading to increased levels of uncompensated care and low health outcomes. The policy analysts caution that to reduce the disruption, the nations can do so by granting periods of grace, simplifying renewal applications, and increasing outreach programs to the vulnerable groups. Two types of sources are used for this analysis, although used in different ways. AI was used to ease preliminary research and recognize general themes. However, it did not factor in the critical legal and fiscal implications associated with lower FMAP rates, the

tax implications for providers, and lost coverage that may be expected to accompany the new documentation burden. In contrast, original research presented the evidence supportive of any findings and policy specific words in context that would lead to more accurate assessment. To sum up, artificial intelligence may be an effective solution in drawing and initial drafting, but not primary research, particularly when it comes to complicating health legislation. But speed is not the prime determinant of credibility, context and precision in public policy analysis. It would have resulted in a flat understanding of Medicaid reform if they tried to do it with just AI. Thus, the merging of AI and a solid, source-based research has become necessary to add up to a thoroughgoing, evidence-based analysis of OBBBA and its broadened implication on the US health care system.

Tabular comparison of AI Generated content Vs my Original Research on OBBBA, 2025

Category	AI Content	Original Article points
Scope & Speed	Fast overview, minimal citation detail	Detailed timelines and fiscal projections from CBO (2025) and CMS (2024) showed specific implementation phases.
Eligibility & Coverage	General tight claim on eligibility	Specific data: 70 % disenrollment (CBO 2025; KFF 2023)
Funding & FMAP	Mentioned reduced funding	Commonwealth Fund (2024) and CMS (2024) documented reduced FMAP rates and capped federal contributions per state.
Provider Payments	Missed alignment with Medicare rates.	Confirmed by AHA (2024) and Commonwealth Fund (2024)
Rural Impact	Not addressed, generalized rural access concerns.	Highlighted hospital instability (AHA 2024)

Source: CBO (2025), CMS (2024), KFF (2023), Commonwealth Fund (2024), and AHA (2024)